

AI Backend

Choose AI model:

- ☐ Offline Demo
- ☒ Gemini (free)

Gemini API Key

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Key set

Policy Documents

Document source:

- ☐ Sample CMS Policies
- ☒ Upload your own

Supports .txt and .pdf files



Healthcare Policy Content Analyzer

AI-powered policy comparison, rule extraction & provider alert generation
| Cotiviti Intern Assessment -- Likitha KB

Analysis completed in 189.6 seconds

Key Changes ...	HIGH Risks	MEDIUM Risks	Denial-Risk R...	Documents C...
31	6	1	34/37	2 P...
	↑ action r...	↑ monitor		

Comparing: 1.pdf vs 2.pdf

Policy Change Summary **Extracted Coding Rules** Provider Alert Policy Diff Viewer

Policy: 1997 DOCUMENTATION GUIDELINES FOR EVALUATION AND MANAGEMENT SERVICES | **Effective:** 1997

CPT Codes

Billing Rules

 'Potential Denial Risk' is AI-inferred from policy language — CMS does not explicitly label rules with this classification. See basis for each rule.

Total Rules	Potential Denial Risk	No Denial Risk
37	34	3

 CPT and ICD-9-CM Code Support 

The CPT and ICD-9-CM codes reported on the health insurance claim form should be supported by the documentation in the medical record.

Potential Denial Risk: Yes (AI-inferred from policy language)

Basis: AI-inferred

 E/M Level Determination

Because the level of E/M service is dependent on two or three key components, performance and documentation of one component (eg, examination) at the highest level does not necessarily mean that the encounter in its entirety qualifies for the highest level of E/M service.

Potential Denial Risk: Yes (AI-inferred from policy language)

Basis: AI-inferred

 Counseling or Coordination of Care Time Factor

In the case of visits which consist predominantly of counseling or coordination of care, time is the key or controlling factor to qualify for a particular level of E/M service.

Potential Denial Risk: Yes (AI-inferred from policy language)

Basis: AI-inferred

 History Qualification Elements

To qualify for a given type of history all three elements in the table must be met. (A chief complaint is indicated at all levels.)

Potential Denial Risk: Yes (AI-inferred from policy language)

Basis: AI-inferred

 Complete ROS Absence of Negative Notation

At least ten organ systems must be reviewed. Those systems with positive or pertinent negative responses must be individually documented. For the remaining systems, a notation indicating all other systems are negative is permissible. In the absence of such a notation, at least ten systems must be individually documented.

Potential Denial Risk: Yes (AI-inferred from policy language)

Basis: In the absence of such a notation, at least ten systems must be individually documented.

 Complete PFSH - 2 areas



At least one specific item from two of the three history areas must be documented for a complete PFSH for the following categories of E/M

services: office or other outpatient services, established patient; emergency department; domiciliary care, established patient; and home care, established patient.

Potential Denial Risk: Yes (AI-inferred from policy language)

Basis: At least one specific item from two of the three history areas must be documented for a complete PFSH

 Complete PFSH - 3 areas



At least one specific item from each of the three history areas must be documented for a complete PFSH for the following categories of E/M services: office or other outpatient services, new patient; hospital observation services; hospital inpatient services, initial care; consultations; comprehensive nursing facility assessments; domiciliary care, new patient; home care, new patient.

Potential Denial Risk: Yes (AI-inferred from policy language)

Basis: At least one specific item from each of the three history areas must be documented for a complete PFSH

 Abnormal Examination Findings Documentation

Specific abnormal and relevant negative findings of the examination of the affected or symptomatic body area(s) or organ system(s) should be documented. A notation of “abnormal” without elaboration is insufficient.

Potential Denial Risk: Yes (AI-inferred from policy language)

Basis: A notation of “abnormal” without elaboration is insufficient.

 Review of Documentation Recorded by Others

To document that the physician reviewed the information, there must be a notation supplementing or confirming the information recorded by others.

Potential Denial Risk: Yes (AI-inferred from policy language)

Basis: To document that the physician reviewed the information, there must be a notation supplementing or confirming the information recorded by others.

 General Multi-System Exam: Problem Focused

Problem Focused Examination – should include performance and documentation of one to five elements identified by a bullet (•) in one or more organ system(s) or body area(s).

Potential Denial Risk: Yes (AI-inferred from policy language)

Basis: Problem Focused Examination – should include performance and documentation of one to five elements identified by a bullet (•) in one or more organ system(s) or body area(s).

 General Multi-System Exam: Expanded Problem Focused

Expanded Problem Focused Examination – should include performance and documentation of at least six elements identified by a bullet (•) in one or more organ system(s) or body area(s).

Potential Denial Risk: Yes (AI-inferred from policy language)

Basis: Expanded Problem Focused Examination – should include performance and documentation of at least six elements identified by a bullet (•) in one or more organ system(s) or body area(s).

 General Multi-System Exam: Detailed



Detailed Examination – should include at least six organ systems or body areas. For each system/area selected, performance and documentation of at least two elements identified by a bullet (•) is expected. Alternatively, a detailed examination may include performance and documentation of at least two elements identified by a bullet (•) in two organ systems or body areas.

least twelve elements identified by a bullet (•) in two or more organ systems or body areas.

Potential Denial Risk: Yes (AI-inferred from policy language)

Basis: Detailed Examination – should include at least six organ systems or body areas. For each system/area selected, performance and documentation of at least two elements identified by a bullet (•) is expected. Alternatively, a detailed examination may include performance and documentation of at least twelve elements identified by a bullet (•) in two or more organ systems or body areas.

 General Multi-System Exam: Comprehensive

Comprehensive Examination – should include at least nine organ systems or body areas. For each system/area selected, all elements of the examination identified by a bullet (•) should be performed, unless specific directions limit the content of the examination. For each area/system, documentation of at least two elements identified by a bullet is expected.

Potential Denial Risk: Yes (AI-inferred from policy language)

Basis: Comprehensive Examination – should include at least nine organ systems or body areas. For each system/area selected, all elements of the examination identified by a bullet (•) should be performed, unless specific directions limit the content of the examination. For each area/system, documentation of at least two elements identified by a bullet is expected.

documented by a patient is expected.

 Single Organ System Exam: Problem Focused

Problem Focused Examination – should include performance and documentation of one to five elements identified by a bullet (•), whether in a box with a shaded or unshaded border.

Potential Denial Risk: Yes (AI-inferred from policy language)

Basis: Problem Focused Examination – should include performance and documentation of one to five elements identified by a bullet (•), whether in a box with a shaded or unshaded border.

 Single Organ System Exam: Expanded Problem Focused

Expanded Problem Focused Examination – should include performance and documentation of at least six elements identified by a bullet (•), whether in a box with a shaded or unshaded border.

Potential Denial Risk: Yes (AI-inferred from policy language)

Basis: Expanded Problem Focused Examination – should include performance and documentation of at least six elements identified by a bullet (•), whether in a box with a shaded or unshaded border.

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Single Organ System Exam: Detailed (Standard vs Eye/Psychiatric)

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Detailed Examination – examinations other than the eye and psychiatric examinations should include performance and documentation of at least twelve elements identified by a bullet (•), whether in a box with a shaded or unshaded border. Eye and psychiatric examinations should include the performance and documentation of at least nine elements identified by a bullet (•), whether in a box with a shaded or unshaded border.

Potential Denial Risk: Yes (AI-inferred from policy language)

Basis: Detailed Examination – examinations other than the eye and psychiatric examinations should include performance and documentation of at least twelve elements identified by a bullet (•), whether in a box with a shaded or unshaded border. Eye and psychiatric examinations should include the performance and documentation of at least nine elements identified by a bullet (•), whether in a box with a shaded or unshaded border.

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Single Organ System Exam: Comprehensive

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Comprehensive Examination – should include performance of all elements identified by a bullet (•), whether in a shaded or unshaded box. Documentation of every element in each box with a shaded border and at least one element in a box with an unshaded border is expected.

Potential Denial Risk: Yes (AI-inferred from policy language)

Basis: Comprehensive Examination – should include performance of all elements identified by a bullet (•), whether in a shaded or unshaded box. Documentation of every element in each box with a shaded border and at least one element in a box with an unshaded border is expected.

 Vital Signs Ancillary Staff Documentation

Measurement of any three of the following seven vital signs: 1) sitting or standing blood pressure, 2) supine blood pressure, 3) pulse rate and regularity, 4) respiration, 5) temperature, 6) height, 7) weight (May be measured and recorded by ancillary staff)

Potential Denial Risk: No — process change only.



Ancillary Staff Vital Signs Recording

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May be measured and recorded by ancillary staff

Potential Denial Risk: No — process change only.



Comprehensive Exam Shaded and Unshaded Border Documentation

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document every element in each box with a shaded border and at least one element in each box with an unshaded border.

Potential Denial Risk: Yes (AI-inferred from policy language)

Basis: document every element in each box with a shaded border and at least one element in each box with an unshaded border.



Detailed Eye Exam Element Threshold

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At least nine elements identified by a bullet.

Potential Denial Risk: Yes (AI-inferred from policy language)

Basis: At least nine elements identified by a bullet.

 Detailed General/ENT/GU Exam Element Threshold

At least twelve elements identified by a bullet.

Potential Denial Risk: Yes (AI-inferred from policy language)

Basis: At least twelve elements identified by a bullet.

 Comprehensive Musculoskeletal Joint, Bone, and Muscle/Tendon Examination

For the comprehensive level of examination, all four of the elements identified by a bullet must be performed and documented for each of four anatomic areas.

Potential Denial Risk: Yes (AI-inferred from policy language)

Basis: For the comprehensive level of examination, all four of the elements identified by a bullet must be performed and documented for each of four anatomic areas.

 Lower Level Musculoskeletal Joint, Bone, and Muscle/Tendon Counting

For the three lower levels of examination, each element is counted separately for each body area. For example, assessing range of motion in two extremities constitutes two elements.

Potential Denial Risk: Yes (AI-inferred from policy language)

Basis: AI-inferred

 Comprehensive Skin Examination in Musculoskeletal Exam

For the comprehensive level, the examination of all four anatomic areas must be performed and documented.

Potential Denial Risk: Yes (AI-inferred from policy language)

Basis: For the comprehensive level, the examination of all four anatomic areas must be performed and documented

be performed and documented.

Lower Level Skin Examination Counting

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For the three lower levels of examination, each body area is counted separately. For example, inspection and/or palpation of the skin and subcutaneous tissue of two extremities constitutes two elements.

Potential Denial Risk: Yes (AI-inferred from policy language)

Basis: AI-inferred

Comprehensive Level Performance and Documentation

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Perform all elements identified by a bullet; document every element in each box with a shaded border and at least one element in each box with an unshaded border.

Potential Denial Risk: Yes (AI-inferred from policy language)

Basis: Perform all elements identified by a bullet; document every element in each box with a shaded border and at least one element in each box with an unshaded border.

 Ancillary Staff Vital Sign Recording

Measurement of any three of the following seven vital signs: 1) sitting or standing blood pressure, 2) supine blood pressure, 3) pulse rate and regularity, 4) respiration, 5) temperature, 6) height, 7) weight (May be measured and recorded by ancillary staff)

Potential Denial Risk: No — process change only.

 Comprehensive Examination Documentation Requirement

Perform all elements identified by a bullet; document every element in each box with a shaded border and at least one element in each box with an unshaded border.

Potential Denial Risk: Yes (AI-inferred from policy language)

Basis: Perform all elements identified by a bullet; document every element in each box with a shaded border and at least one element in each box with an unshaded border.

 Problem Focused Exam Requirement



One to five elements identified by a bullet.

Potential Denial Risk: Yes (AI-inferred from policy language)

Basis: One to five elements identified by a bullet.

 Expanded Problem Focused Exam Requirement



At least six elements identified by a bullet.

Potential Denial Risk: Yes (AI-inferred from policy language)

Basis: At least six elements identified by a bullet.

 Detailed Psychiatric Exam Requirement

At least nine elements identified by a bullet.

Potential Denial Risk: Yes (AI-inferred from policy language)

Basis: At least nine elements identified by a bullet.

 Detailed Neurological/Respiratory Exam Requirement

At least twelve elements identified by a bullet.

Potential Denial Risk: Yes (AI-inferred from policy language)

Basis: At least twelve elements identified by a bullet.

 Comprehensive Examination Anatomic Area Requirements

For the comprehensive level, the examination of at least eight anatomic

For the comprehensive level, the examination of at least eight anatomic areas must be performed and documented.

Potential Denial Risk: Yes (AI-inferred from policy language)

Basis: For the comprehensive level, the examination of at least eight anatomic areas must be performed and documented.

Medical Decision Making Level Qualification

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To qualify for a given type of decision making, two of the three elements in the table must be either met or exceeded.

Potential Denial Risk: Yes (AI-inferred from policy language)

Basis: To qualify for a given type of decision making, two of the three elements in the table must be either met or exceeded.

Old Records and Additional History Documentation Elaboration

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Relevant findings from the review of old records, and/or the receipt of additional history from the family, caretaker or other source to supplement that obtained from the patient should be documented. If there is no relevant information beyond that already obtained, that fact should be documented. A notation of 'Old records reviewed' or 'additional history obtained from family' without elaboration is insufficient.

Potential Denial Risk: Yes (AI-inferred from policy language)

Basis: A notation of “Old records reviewed” or “additional history obtained from family” without elaboration is insufficient.

E/M Reporting Based on Counseling and/or Coordination of Care

In the case where counseling and/or coordination of care dominates (more than 50%) of the physician/patient and/or family encounter (face-to-face time in the office or other or outpatient setting, floor/unit time in the hospital or nursing facility), time is considered the key or controlling factor to qualify for a particular level of E/M services.

Potential Denial Risk: Yes (AI-inferred from policy language)

Basis: If the physician elects to report the level of service based on counseling and/or coordination of care, the total length of time of the encounter (face-to-face or floor time, as appropriate) should be documented and the record should describe the counseling and/or activities to coordinate care.

Reimbursement

Participating Rate	Non-Participating Rate	Telehealth Parity
		Active

Excluded Services

- subsequent hospital care
- follow-up inpatient consultations